

MedEd-HalluScore Rubric and Reviewer Checklist v0.2

Scoring Rubric

Each dimension is scored from 0 to 3.

1. Medical Factuality

0: No apparent factual error.

1: Minor factual issue with low educational impact.

2: Moderate factual error that could confuse learners.

3: Severe factual error that could teach unsafe or false clinical knowledge.

2. Internal Clinical Consistency

0: Clinically coherent throughout.

1: Minor inconsistency that does not change the main teaching point.

2: Relevant inconsistency that affects interpretation.

3: Severe contradiction between case elements.

3. Critical Information Omissions

0: No relevant omission.

1: Minor omission.

2: Omission that weakens clinical reasoning.

3: Safety-critical omission.

4. Clinical Reasoning Risk

0: Reasoning is appropriate and educationally sound.

- 1: Mild oversimplification.
- 2: Incomplete or biased reasoning.
- 3: Dangerous reasoning pattern.

5. Educational Safety

- 0: Suitable for educational use after routine review.
- 1: Requires minor corrections.
- 2: Requires substantial revision.
- 3: Not recommended without full rewrite.

6. Transparency and Verifiability

- 0: Claims are clear and verifiable.
- 1: Some claims lack context.
- 2: Many claims are vague, overconfident, or hard to verify.
- 3: Technical claims appear invented or nontraceable.

Risk Classification

- 0-3: Low Risk
- 4-8: Moderate Risk
- 9-13: High Risk
- 14-18: Critical Risk

Reviewer Checklist

Clinical accuracy:

- Are diagnoses, mechanisms, and treatments medically accurate?
- Are medication classes, indications, and contraindications correct?
- Are red flags handled appropriately?

Clinical coherence:

- Do symptoms, signs, tests, diagnosis, and management align?
- Are timelines plausible?
- Are lab values and imaging findings interpreted correctly?

Missing information:

- Are age and relevant background included?
- Are vital signs included when clinically important?
- Are key labs, imaging, or risk factors included?
- Are emergency or safety-critical features addressed?

Reasoning quality:

- Does the explanation avoid premature closure?
- Are differential diagnoses considered when appropriate?
- Does the case teach a sound clinical reasoning pathway?

Educational safety:

- Could a learner take away an unsafe rule or concept?
- Does the case need minor edits, major revision, or rejection?
- Is the case clearly limited to educational use?

Verifiability:

- Are claims specific enough to verify?
- Are uncertain or context-dependent statements appropriately qualified?
- Are nonexistent guidelines, biomarkers, or unsupported claims avoided?